

Class 5 – Shoulder Fracture: Patient Care Guide

A clear guide for patients with a shoulder fracture — a break in one of the bones that form the shoulder: the collarbone (clavicle), the shoulder blade (scapula), or the top of the upper arm bone (proximal humerus). It covers causes, symptoms, diagnosis, treatment, healing, exercises, warning signs, and common questions.

Medical disclaimer: This document is an educational reference created for a patient-support knowledge base and for use in a retrieval-augmented (RAG) AI assistant. It is NOT a substitute for professional medical advice, diagnosis, or treatment. Healing times, medicines, and exercises differ for every person. Always follow the instructions of your own doctor, orthopaedic surgeon, or physiotherapist. In an emergency, contact local emergency services immediately.

1. Introduction

A shoulder fracture is a break in one of the bones that make up the shoulder. The shoulder is where the arm meets the body, and it allows the widest range of movement of any joint. A break here is also called a broken shoulder.

Which bones are involved: the clavicle (collarbone) at the front, the scapula (shoulder blade) at the back, and the proximal humerus (the ball at the top of the upper arm bone). Clavicle and proximal humerus fractures are the most common; scapula fractures are rarer and usually follow high-energy trauma.

Common causes: falls onto the shoulder or an outstretched hand, road and cycling accidents, sports collisions, and direct blows. How common it is: clavicle fractures are very common in young, active people, while proximal humerus fractures are common in older adults with osteoporosis.

Who is at higher risk: athletes in contact and wheel sports, older adults with weak bones, and people in high-impact accidents. Severity ranges from an undisplaced crack (mild) to a displaced, multi-fragment, or open fracture (severe).

Types of Fractures Affecting This Bone

A shoulder fracture can happen in several patterns. Common fracture types include:

- Hairline fracture (a very thin, small crack in the bone).
- Stress fracture (a tiny crack caused by repeated force or overuse).
- Greenstick fracture (the bone bends and cracks on one side; common in children).
- Closed fracture (the bone breaks but the skin stays intact).
- Open (compound) fracture (the broken bone breaks through the skin).
- Comminuted fracture (the bone breaks into three or more pieces).
- Spiral fracture (a break that twists around the bone, often from a twisting injury).
- Impacted fracture (the broken ends are driven into each other).
- Transverse fracture (a straight break across the bone).
- Oblique fracture (a slanted or angled break across the bone).

The most common shoulder fractures are mid-shaft clavicle (collarbone) fractures and proximal humerus fractures near the shoulder ball; scapula fractures usually mean a very forceful injury.

2. Anatomy

The shoulder is a ball-and-socket joint supported by several bones, muscles, and the rotator cuff.

- Bones involved: the clavicle (collarbone), the scapula (shoulder blade, including the socket called the glenoid), and the proximal humerus (the ball).
- Nearby muscles: the deltoid gives the shoulder its shape and lifts the arm; the rotator cuff muscles keep the ball centred in the socket; large back and chest muscles also attach here.
- Tendons: the rotator cuff tendons wrap the top of the humerus and are essential for raising and rotating the arm.
- Ligaments: strong ligaments connect the clavicle to the scapula and stabilise the joint.
- Blood vessels: major vessels to the arm (the subclavian and axillary arteries) pass near the shoulder and can rarely be injured in severe fractures.
- Nerves: the brachial plexus (the bundle of nerves to the whole arm) and the axillary nerve lie close to the shoulder and can be affected, causing arm weakness or numbness.

Because the rotator cuff and deltoid pull on the bones, and because major nerves and vessels pass nearby, shoulder fractures can affect both movement and, rarely, the nerve and blood supply to the arm. Good alignment and rehabilitation are key to regaining the shoulder's wide range of movement.

3. Common Causes

Most fractures happen when a force is stronger than the bone can handle. The main causes are:

- Falls: slipping, tripping, or falling onto an outstretched hand or arm — the most common cause overall.
- Road traffic accidents: high-speed impacts from car, motorbike, or cycle crashes.
- Sports injuries: contact sports, skating, cycling, gymnastics, and falls during play.
- Workplace accidents: falls from height, machinery injuries, or heavy objects.
- Direct trauma: a hard blow or hit directly on the bone.
- Twisting injuries: the limb turns while the hand or body stays fixed.
- Heavy object injuries: something heavy falling onto the limb.
- Osteoporosis: weak, thin bones that break easily, especially in older adults.
- Bone tumors: growths that weaken the bone (pathological fracture).
- Repetitive stress: repeated overuse leading to stress fractures over time.

For the shoulder specifically, a direct fall onto the shoulder or onto an outstretched hand is the most common cause.

4. Symptoms

Common signs and symptoms of a shoulder fracture include:

- Pain around the shoulder, worse with any arm movement.
- Swelling and bruising over the collarbone or shoulder.

- A bump, deformity, or drooping of the shoulder (common with clavicle fractures).
- Inability to lift or rotate the arm.
- A grinding or cracking feeling with movement.
- Numbness or tingling in the arm or hand (possible nerve involvement).
- Holding the arm close to the body for support.
- The skin over the collarbone looking tented or stretched.
- In an open fracture, bone may be visible through a wound.

Emergency warning signs: a cold, pale, or blue hand, no pulse at the wrist, weakness or numbness of the whole arm, skin about to break over a sharp bone end, difficulty breathing after the injury, or bone through the skin. Seek emergency care immediately.

5. Diagnosis

Doctors confirm a fracture using a physical check and imaging tests. Each test gives different information.

Physical Examination and Medical History

The doctor asks how the injury happened and checks for pain, swelling, bruising, deformity, tenderness, movement, pulse, skin colour, and sensation. Telling the doctor about past fractures, osteoporosis, medicines, and other illnesses helps plan the right treatment.

Imaging Tests

- X-ray: the first and main test. It shows most fractures, the exact location, and whether the bone has moved out of place (displacement).
- CT scan: gives detailed 3-D images. Used for complex breaks, joint involvement, or when the X-ray is unclear, to plan surgery.
- MRI: shows soft tissues, bone bruising, and hairline or stress fractures that may not appear on an X-ray. Also checks nearby ligaments, tendons, and nerves.
- Ultrasound: sometimes used, especially in children, to look at bone surfaces and soft-tissue swelling without radiation.
- Bone scan: can detect very small stress fractures or hidden fractures that other tests miss, by showing areas of increased bone activity.

For a shoulder fracture, X-rays from several angles are the main test and show clavicle and humerus fractures well. A CT scan is often needed for scapula and proximal humerus fractures to see the fragments and joint surface before deciding on surgery. The doctor also checks breathing and the nerves and pulses of the arm.

6. Treatment Options

Treatment depends on the type of break, how far the bone has moved, and your age and health. It falls into non-surgical and surgical options.

Non-Surgical Treatment

Most clavicle fractures and many undisplaced proximal humerus and scapula fractures heal well without surgery.

- Sling or shoulder immobiliser: the main treatment; it supports the arm and rests the shoulder while the bone heals.
- Figure-of-eight brace: sometimes used for clavicle fractures to hold the shoulders back.
- Ice therapy: reduces pain and swelling in the first days.
- Elevation and support: keeping the arm supported eases discomfort.
- Pain medications: paracetamol for pain relief.
- Anti-inflammatory medicines: such as ibuprofen, if approved by your doctor.
- Early gentle movement: pendulum and hand/elbow exercises to prevent stiffness once advised.

When recommended: non-surgical care suits stable fractures where the bone is in a good position. Advantages: no surgery, lower infection risk, and no anaesthesia. Limitations: needs a well-set position, the cast can cause stiffness, and it may not hold badly displaced or unstable fractures.

Surgical Treatment

Surgery is used when the bone is badly out of place, broken into many pieces, involves a joint, is an open (compound) fracture, or fails to heal with a cast. The aim is to put the pieces back in the correct position and hold them still while they heal.

- ORIF (Open Reduction Internal Fixation): the surgeon opens the skin, sets the bone in the right position (reduction), and fixes it with metal implants.
- Plates: flat metal supports screwed along the bone to hold the pieces together.
- Screws: hold fragments together, alone or with a plate.
- Pins (K-wires): thin wires that hold small pieces, often used in fingers and in children; usually removed later.
- Rods (intramedullary nails): metal rods placed inside the hollow centre of a long bone to support it.
- External fixator: a frame outside the body connected to the bone by pins; used for severe, open, or badly contaminated fractures.
- Bone graft: extra bone (from your own body or a donor) added to help healing when there is bone loss or slow healing.

Indications (when surgery is needed): badly displaced or shortened clavicle fractures, displaced proximal humerus fractures, fractures breaking through the skin, fractures with nerve or vessel injury, and certain scapula/glenoid (socket) fractures. Clavicles are usually fixed with a plate and screws; proximal humerus fractures may need plates, screws, or a shoulder replacement in severe cases.

Recovery Expectations and Risks After Surgery

- Recovery: a protective cast, splint, or sling is often used after surgery, followed by physiotherapy. Implants usually stay in place unless they cause problems.
- Risks: infection, bleeding, nerve or blood vessel injury, implant irritation or loosening, stiffness, blood clots, and reaction to anaesthesia.
- Most people regain good function, but full recovery takes weeks to months and depends on the injury and following rehabilitation.

7. Healing Timeline

Healing time varies by age, general health, fracture type, and how well instructions are followed. The table gives typical ranges.

Group	Typical healing / recovery
Children	Clavicle often unites in 3–4 weeks; children heal and remodel quickly.
Adults	Clavicle about 6–8 weeks; proximal humerus about 6–12 weeks for union.
Older adults	8–12 weeks or more; healing slower with osteoporosis.
Athletes	Union in 6–12 weeks; return to contact sport often 3–6 months.

Stages of healing: initial healing (inflammation and soft callus) in the first weeks, bone union (hard callus bridges the break), and functional recovery (regaining strength and movement). Regaining full overhead reach and rotation is often the last and slowest part of recovery.

- Return to work: desk work in 1–3 weeks; heavy/overhead work often 10–12 weeks or after clearance.
- Return to sports: non-contact around 8–12 weeks; contact sports usually 3–6 months.
- Full recovery: 3–6 months for most; shoulder stiffness can take longer to settle.

Factors that delay healing: smoking, alcohol, poor nutrition, diabetes, osteoporosis, infection, poor blood supply, older age, certain medicines (such as steroids), and not resting the injured limb.

8. Recovery Stages

Knowing what to expect at each stage helps you feel reassured. Your own timeline may differ slightly.

Stage	What usually happens
Week 1	Pain and swelling peak; arm rested in sling; move fingers, wrist, and elbow.
Week 2	Pain easing; begin gentle pendulum shoulder movement if advised.
Week 3–4	Bone knitting; sling continues; gentle assisted shoulder movement progresses.
Week 5–8	Bone uniting; more active shoulder movement; physiotherapy builds range.
Month 3	Bone usually united; focus on full range and strength.
Month 6	Most movement and strength restored; return to demanding activity.

9. Daily Care Instructions

- Keep any brace or dressing dry: cover it while bathing to protect the skin.
- Sleeping position: many people sleep semi-upright, propped on pillows or in a recliner, for the first weeks; keep the arm supported in the sling.
- Bathing: use a waterproof cover or sponge-bathe; keep the arm supported and still.
- Clothing: wear loose, front-opening clothes; dress the injured arm first.
- Walking: normal walking is fine; keep the arm in the sling for support.

- Driving: avoid driving until you are out of the sling and can control the car safely.
- Returning to office: light desk work is often possible within 1–3 weeks.
- Returning to school: children can attend once comfortable, avoiding sport and PE.
- Household activities: avoid lifting, reaching overhead, and carrying with the injured arm.

10. Pain Management

Some pain is normal after a fracture, especially in the first days. These safe, simple methods help control it.

- Ice packs: wrap ice in a cloth and apply for 15–20 minutes several times a day to reduce pain and swelling (never put ice directly on the skin).
- Elevation: keep the injured limb raised above the level of the heart to reduce throbbing and swelling.
- Rest: avoid using the injured limb for gripping, lifting, or bearing weight.
- Painkillers: paracetamol (acetaminophen) is commonly used. Anti-inflammatory medicines such as ibuprofen may be advised. Take only the dose your doctor approves.
- Safe positioning: support the limb on a pillow and avoid letting it hang down.
- Relaxation techniques: slow breathing, distraction, and gentle music can reduce the sensation of pain.

When pain is abnormal: pain that suddenly becomes severe, is not relieved by medicine, comes with numbness, a cold or blue hand, or a very tight cast may signal a serious problem such as compartment syndrome or poor blood supply. Seek medical help urgently.

11. Diet for Bone Healing

Good nutrition speeds up bone healing. The nutrients below all play a role. Try to eat a balanced diet that includes them every day, and follow any special diet advice from your doctor or dietitian.

Key Nutrients for Bone Healing

Nutrient	Why it helps & daily intake	Vegetarian sources	Non-vegetarian sources
Calcium	Main mineral that builds and repairs bone. About 1000–1200 mg/day for adults.	Milk, curd, paneer, cheese, ragi, tofu, green leafy vegetables, sesame seeds	Small fish eaten with bones (sardines), dairy
Vitamin D	Helps the body absorb calcium. About 600–800 IU/day (more if deficient).	Sunlight, fortified milk and cereals, mushrooms	Egg yolk, fatty fish (salmon, mackerel), fish liver oil
Protein	Builds the collagen framework of bone. About 1.0–1.5 g per kg body weight.	Dal, beans, chickpeas, rajma, soya, paneer, tofu, nuts	Eggs, chicken, fish, lean meat
Vitamin C	Needed to make collagen for bone repair. About 65–90 mg/day.	Oranges, lemon, amla, guava, capsicum, tomato, broccoli	(Mainly from plant foods)

Nutrient	Why it helps & daily intake	Vegetarian sources	Non-vegetarian sources
Vitamin K	Helps bind calcium into bone. About 90–120 mcg/day.	Spinach, kale, broccoli, cabbage, green leafy vegetables	Eggs, some dairy
Magnesium	Supports bone structure and vitamin D use. About 310–420 mg/day.	Nuts, seeds, whole grains, bananas, dark leafy greens	Fish
Phosphorus	Works with calcium to harden bone. About 700 mg/day.	Dairy, beans, lentils, whole grains, nuts	Meat, fish, poultry, eggs
Zinc	Helps bone cells grow and repair. About 8–11 mg/day.	Whole grains, legumes, seeds, nuts	Meat, shellfish, eggs
Omega-3 fats	Reduce inflammation and support healing.	Flaxseed, chia seeds, walnuts, soybean oil	Fatty fish (salmon, mackerel, sardines)
Collagen	Forms the flexible base of bone. Supported by protein + vitamin C.	Body makes it from protein-rich plant foods + vitamin C	Bone broth, chicken, fish, eggs

Drink enough water and, if your doctor recommends, take calcium and vitamin D supplements. Do not take high-dose supplements on your own without medical advice.

12. Foods to Avoid

Some foods and habits slow bone healing by reducing calcium absorption, increasing inflammation, or harming blood supply. Limit or avoid the following while your fracture heals.

- Alcohol: slows new bone formation and reduces calcium and vitamin D use. Best avoided completely during healing.
- Smoking and tobacco: reduce blood flow to the bone and are a leading cause of delayed healing and nonunion. Stopping greatly improves healing.
- Sugary foods: too much sugar can increase inflammation and displace healthy foods.
- Soft drinks and colas: high phosphoric acid and sugar may reduce calcium balance.
- Excess caffeine: large amounts of coffee, tea, or energy drinks can reduce calcium absorption; keep intake moderate.
- Processed and packaged food: often high in salt and low in nutrients.
- High sodium (salt) intake: makes the body lose more calcium in urine.
- Junk and deep-fried food: high in unhealthy fat and low in bone-building nutrients.

In short, a fresh, home-cooked, balanced diet with less salt, sugar, alcohol, and no smoking gives your bone the best chance to heal quickly.

13. Physical Therapy and Rehabilitation Exercises

Physiotherapy restores movement, strength, and function. Start only when your doctor allows, and stop any exercise that causes sharp or increasing pain. Below are typical stages — your therapist will

tailor them to you.

Early Stage

- Finger, wrist, and elbow movements — purpose: keep the arm mobile while the shoulder rests. Repetitions: gentle, 10 each, several times daily.
- Pendulum exercises (bending forward and letting the arm swing gently) — purpose: early shoulder motion without straining. Precaution: only when your doctor allows.

Middle Stage

- Assisted shoulder raises using the good arm or a stick — purpose: regain range safely. Repetitions: 10 slow reps. Precaution: stay within a comfortable range.
- Shoulder blade squeezes — purpose: improve posture and shoulder control.
- Gentle rotation exercises — purpose: restore turning movement.

Advanced Stage

- Resistance-band rotator-cuff and shoulder exercises — purpose: rebuild strength. Repetitions: 10–15, 2–3 sets. Precaution: start light.
- Overhead reaching and strengthening — purpose: restore full function.
- Sport-specific drills and gradual loading — purpose: safe return. Precaution: increase slowly; stop if pain returns.

General exercise precautions: move slowly, never push through sharp pain, stop if you feel numbness or dizziness, and follow the exact repetitions your therapist advises. When to stop: stop and call your therapist if pain worsens, swelling increases, or movement suddenly decreases.

14. Activities to Avoid During Healing

Avoiding certain activities protects the healing bone and implants.

Activity	How long to avoid (typical)
Lifting or carrying with the injured arm	Until healed — often 8–12 weeks
Reaching or working overhead	About 10–12 weeks or until cleared
Push-ups, pull-ups, weight training	About 3 months or until cleared
Contact sports and collisions	About 3–6 months
Cycling/riding (fall risk on the shoulder)	About 8–12 weeks
Carrying a bag on the injured shoulder	Until full strength returns (about 3 months)

15. Possible Complications

Most fractures heal well, but knowing the possible problems helps you spot them early. Report any of these to your doctor.

- Delayed union: the bone heals more slowly than expected. Prevented by good nutrition, not smoking, and following restrictions.
- Nonunion: the bone fails to join. Signs are ongoing pain and movement at the break site; may need surgery or a bone graft.
- Malunion: the bone heals in the wrong position, causing deformity or reduced function; sometimes needs correction.
- Infection: mainly with open fractures or after surgery. Signs are fever, redness, warmth, swelling, pus, or bad smell.
- Stiffness: nearby joints become stiff from being kept still; prevented by physiotherapy and gentle movement as advised.
- Nerve injury: causes numbness, tingling, or weakness; often improves but should be checked.
- Blood vessel injury: rare but serious; causes a cold, pale, or blue limb and needs emergency care.
- Chronic (long-lasting) pain: pain that continues after healing; managed with therapy and pain specialists.
- Arthritis: joint fractures can lead to wear-and-tear arthritis years later.
- Reduced mobility: lasting loss of full movement, reduced by good rehabilitation.
- Complex Regional Pain Syndrome (CRPS): an uncommon condition with severe pain, swelling, skin colour and temperature changes; early treatment gives the best result.

16. Red Flag Symptoms — Go to Hospital Immediately

Get emergency medical help right away if you notice any of the following. These can mean the blood supply, nerves, or the healing is in danger.

- Severe or rapidly increasing swelling.
- Fingers (or toes) turning blue, grey, or very pale.
- The hand or limb feels cold to touch.
- Numbness or loss of feeling that does not go away.
- Fever or chills (a sign of infection).
- Bleeding that will not stop.
- Severe pain that is not relieved by rest, elevation, or prescribed medicine.
- The cast becomes too tight or the limb feels squeezed.
- The cast breaks, cracks, or becomes loose.
- Pus, discharge, or fluid coming from under the cast or a wound.
- A bad smell coming from the cast or wound.
- Bone is visible through the skin (open fracture).
- Difficulty breathing, chest pain, or coughing after the injury (a possible blood clot or fat embolism — a rare but serious emergency).

Compartment syndrome warning: severe deep pain, tightness, numbness, and pain when the fingers are gently stretched can signal dangerous pressure build-up. This is an emergency — do not wait.

17. Follow-Up Schedule

Regular follow-up lets the doctor check that the bone is healing in the right position. A typical schedule is below.

Visit	What the doctor usually checks
Week 1	Check swelling, sling fit, skin over the bone, arm nerves and pulses; X-ray.
Week 2	Confirm position held; check the wound if operated; start gentle movement.
Week 4	X-ray for early healing; progress assisted shoulder movement.
Week 6	Check union; advance active movement and strengthening.
Week 12	Confirm union; assess range, strength, and plan return to sport/heavy work.

18. Returning to Normal Life

These are general guides for when people usually return to activities. Always get your doctor's clearance first.

Activity	Typical time to return
School	A few days to 1–2 weeks (no sport/PE)
Office (desk job)	1–3 weeks
Driving	After the sling is off and control is safe — often 6–8 weeks
Gym (upper body)	About 3 months, building up gradually
Swimming	After healing and good range — about 10–12 weeks
Cycling	Once stable and low fall-risk — about 8–12 weeks
Running	Often 4–6 weeks once comfortable
Cricket / Football	About 3–6 months with clearance
Weight lifting	About 3–4 months, increasing load slowly

19. Prevention of Future Fractures

After one fracture, it is worth protecting your bones for the future. These steps lower the chance of breaking a bone again.

- Bone-strengthening exercises: regular resistance and muscle-strengthening activity keeps bones dense.
- Weight-bearing exercise: walking, jogging, dancing, and stair climbing build bone strength.
- Balance training: yoga, tai chi, and balance drills reduce falls, especially in older adults.
- Vitamin D: from sunlight, diet, or supplements as advised, to help absorb calcium.
- Calcium intake: eat enough calcium-rich foods every day.
- Protective equipment: wrist guards, elbow pads, and helmets during sports and skating.
- Safe workplace habits: follow safety rules, use guards, and prevent falls from height.

- Sports safety: warm up, use correct technique, and wear the right gear.
- Fall prevention: good footwear, adequate lighting, and vision checks.
- Home safety tips: remove loose rugs, add grab bars, keep floors dry and clutter-free.
- Osteoporosis screening: a bone density (DEXA) scan for older adults and high-risk people, with treatment if bones are weak.

20. Frequently Asked Questions (FAQ)

Q1. Can I move my fingers?

Yes. Move your fingers, wrist, and elbow gently while the shoulder rests in the sling. This keeps the arm from getting stiff and helps swelling.

Q2. Can I sleep on the injured shoulder?

No. Many people sleep better semi-upright, propped on pillows, with the arm supported. Avoid lying on the injured side.

Q3. Can I remove the sling?

Only as your doctor advises. The sling usually stays on most of the time early on, and is removed for gentle exercises once permitted.

Q4. When can I drive?

Usually after you are out of the sling and can control the car safely — often around 6–8 weeks. Confirm with your doctor.

Q5. Can I exercise?

Gentle hand, wrist, elbow, and lower-body exercise is fine early. Shoulder strengthening comes later, on your doctor's advice.

Q6. Can I lift weights?

Not until the bone heals and you are cleared — usually about 3–4 months, starting light.

Q7. Is the bump on my collarbone permanent?

A visible bump after a clavicle fracture is common and often remains, but it usually does not affect function.

Q8. Is swelling and bruising normal?

Yes, in the first days to weeks. Report severe swelling, a cold or blue hand, or spreading arm numbness urgently.

Q9. Is itching under the brace normal?

Yes, mild itching is common. Do not insert objects to scratch; cool air can help.

Q10. Can I travel?

Short travel is fine once pain is controlled and the arm is supported. Ask before long journeys.

Q11. Can I fly?

Usually yes; tell your doctor, especially soon after injury, as swelling can increase.

Q12. Can I get the brace or dressing wet?

Keep it dry with a waterproof cover during bathing to protect the skin and any wound.

Q13. How long should I wear the sling?

Often about 2–6 weeks depending on the fracture and your doctor's plan.

Q14. When should I see a doctor urgently?

For a cold or blue hand, no wrist pulse, whole-arm weakness or numbness, skin breaking over the bone, breathing trouble, or a wound with discharge or fever.

Q15. Why is my shoulder so stiff?

After weeks in a sling the shoulder stiffens easily. This is common and improves with physiotherapy; overhead movement returns slowly.

Q16. Will I need surgery?

Not always. Most clavicle and many shoulder fractures heal in a sling. Badly displaced, open, or nerve-injury fractures often need surgery.

Q17. Do the plates or screws need removal?

Usually they can stay for life. Clavicle plates are sometimes removed later if they irritate the skin.

Q18. My whole arm feels weak or numb — is that serious?

This can mean nerve involvement (the brachial plexus or axillary nerve). Tell your doctor promptly; it often improves but must be checked.

Q19. Is some pain normal weeks later?

Mild aching during healing and physiotherapy is normal. Sharp or worsening pain should be reviewed.

Q20. What foods help healing?

Calcium, vitamin D, protein, and vitamin C support bone repair. Avoid smoking and alcohol, which slow healing.

Q21. Can I go to work?

Desk work is often possible in 1–3 weeks. Heavy or overhead work usually needs 10–12 weeks or clearance.

Q22. Will my arm be weak after the sling comes off?

Yes, temporary weakness and stiffness are expected. Physiotherapy restores strength and movement.

Q23. Can I play sports again?

Yes, most people do. Non-contact around 8–12 weeks and contact sports around 3–6 months, after clearance.

Q24. Can I raise my arm overhead again?

Most people regain overhead movement with physiotherapy, though it is often the last motion to fully return.

Q25. How can I prevent another shoulder fracture?

Strengthen bones and muscles, prevent falls, wear protective gear in sport, and get osteoporosis screening if at risk.

Q26. Can I use my hand for eating and phone use?

Yes, gentle hand use for light tasks is usually fine if it does not hurt the shoulder.

21. Metadata (for Retrieval)

Structured tags to help the AI assistant retrieve this document accurately.

Field	Value
Fracture Name	Shoulder Fracture
Bones Involved	Clavicle (collarbone), Scapula (shoulder blade), Proximal humerus
Body Region	Upper limb (shoulder girdle)
Common Causes	Falls onto shoulder/outstretched hand, road & cycling accidents, sports collisions, direct trauma, osteoporosis
Symptoms	Pain, swelling, deformity/bump, unable to lift arm, numbness, tented skin
Treatment Options	Sling, shoulder immobiliser, figure-of-eight brace; ORIF with plate/screws, shoulder replacement in severe proximal fractures
Healing Duration	Clavicle 6–8 weeks; proximal humerus 6–12 weeks; full recovery 3–6 months
Rehabilitation Duration	8–20 weeks of physiotherapy; stiffness may take longer
Emergency Signs	Cold/blue hand, absent pulse, whole-arm weakness, skin at risk, breathing difficulty, open bone
Keywords	shoulder fracture, broken shoulder, clavicle fracture, collarbone fracture, proximal humerus fracture, scapula fracture
Synonyms	Broken shoulder, broken collarbone, cracked shoulder blade
Related Injuries	Humerus fracture, rotator cuff injury, shoulder dislocation, AC joint injury